



DATE OF DISCHARGE:

[REDACTED]

SEX: M

DOCTOR: _____

DATE OF SERVICE:

Pathology Consultation Report

Final

CASE:

ANATOMIC PATHOLOGY DIAGNOSIS:

- A - Bladder, anterior wall and dome, transurethral excision
- High grade papillary urothelial carcinoma
 - Invasive into muscularis propria
 - No lymphovascular or perineural invasion is identified

- B - Kidney, left renal pelvis, biopsy
- Low grade papillary urothelial carcinoma
 - Invasion cannot be assessed

1CD-0-3

carcinoma, papillary urothelial 8130/3

Site: bladder, nos C67.9

hw
9/28/12

PATHOLOGIST:

I have reviewed this material and confirm the report. Released by
electronic signature on:

MATERIAL:

- A. Bladder tumor, anterior wall and dome
B. Left renal pelvis biopsy

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
IPAA Discrepancy		
Prior Malignancy History		
Local/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MB	
Date Reviewed:	7/23/12	

8/27/12

HISTORY:

... male with bladder tumor.

GROSS:

Two specimens are received in formalin each properly labeled and identified.

A. The first specimen is designated "bladder tumor, anterior wall and dome". It consists of multiple pieces of fibroelastic tissue measuring 4.5 cm x 4.5 cm x 1.5 cm in aggregate. It is entirely submitted in A1-A8.

B. The second specimen is designated "left renal pelvis biopsy". It consists of one piece of tissue measuring 0.1 cm. It is entirely submitted in B1.

Patient: [REDACTED]

NAME : [REDACTED]

ACCT NBR: [REDACTED]

MICROSCOPIC:

A - The bladder resection contains extensive exophytic and invasive high grade papillary urothelial carcinoma infiltrating in nests and cords with areas of clear cell morphology. Invasion into muscularis propria is identified. Areas of necrosis and calcification are also seen. Lymphovascular invasion is difficult to evaluate due to extensive cautery artifact and the presence of cleft-like spaces around some tumor nests, but no definite lymphovascular invasion is seen. No perineural invasion is seen.

B - The left renal pelvis biopsy consists of a small fragment of papillary urothelium with enlarged nuclei and rare mitotic figures. The pleomorphism is less marked than in the bladder specimen and there is a lower nucleus to cytoplasm ratio. No lamina propria is present to allow for evaluation of invasion.

INTRADEPARTMENTAL CONSULTATION:

[REDACTED]

[REDACTED]

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SURGICAL PATHOLOGY

DICTATED BY:

This report was verified electronically.

Patient: [REDACTED]

Encounter: [REDACTED]

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DATE OF DISCHARGE:

[REDACTED]
SEX: M DOB:
DOCTOR:

DATE OF SERVICE:

Pathology Consultation Report

Final

CASE:

CLINICAL INFORMATION:

HISTORY: urinary bladder - papillary urothelial carcinoma,
high grade.

MATERIAL:

Date Received:

RENAL PELVIS BRUSHING, LEFT

PREPARATION:

Brush(es) received in
1 ThinPrep(s) Prepared
1 Smear(s) Received

ADEQUACY:

Satisfactory for evaluation.

GENERAL DIAGNOSTIC CATEGORY:

Suspicious for malignancy.

DESCRIPTIVE DIAGNOSIS:

Numerous urothelial cells with low nuclear atypia. Some clusters of
urothelial cells with papillary appearance
The findings are suspicious for low grade papillary urothelial
carcinoma.

CYTOTECHNOLOGIST:

VERIFIED BY:

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electronic signature on:

NAME : [REDACTED]

ACCT NBR: [REDACTED]

[REDACTED]

[REDACTED]

Page 1 of 1

CYTOPATHOLOGY

DICTATED BY:

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Patient: [REDACTED]

Encounter: [REDACTED]

Page 2 of 6

DATE OF DISCHARGE:

SEX: M

DOCTOR:

DATE OF SERVICE:

Pathology Consultation Report

Final

CASE:

CLINICAL INFORMATION:

HISTORY: urinary bladder - papillary urothelial carcinoma,
high grade

MATERIAL:

Date Received:

RENAL PELVIS WASHING, LEFT , POST BRUSHING

PREPARATION:

Volume: 0.3 cc

Color: Pink/tan

Consistency: Cloudy

1 ThinPrep(s) Prepared

ADEQUACY:

Satisfactory for evaluation.

GENERAL DIAGNOSTIC CATEGORY:

Negative for malignancy.

DESCRIPTIVE DIAGNOSIS:

No cells of high grade urothelial dysplasia / carcinoma are seen.

CYTOTECHNOLOGIST:

VERIFIED BY:

I have reviewed this material and confirm the report. Released by
electronic signature on:

Patient

Encounter

Page 3 of 6

NAME : [REDACTED]

ACCT NBR: [REDACTED]

[REDACTED]

Page 1 of 1

CYTOPATHOLOGY

DICTATED BY:

This report was verified electronically.

Patient [REDACTED]

Encounter [REDACTED]

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